BASELINE → TREATMENT → FOLLOW-UP

Required and conditional checks across your treatment

Required checks are shown explicitly. electrocardiogram (ECG), additional urine tests and anticoagulant tests apply only when indicated. Scans and tumour markers follow your individual cancer plan; this page does not prescribe a universal surveillance interval. Confirm who books and reviews each test.

Check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver	Required baseline blood count, kidney and liver tests	Required before each cycle; results reviewed before treatment	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed
dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / fluoropyrimidine safety	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
Blood pressure / urine / healing	Required baseline blood pressure (BP) and urine protein	Urine dipstick each cycle; blood pressure monitored during treatment	Report severe headache, bleeding, swelling, reduced urine or poor healing	A raised urine protein/creatinine ratio may require a 24-hour collection; team interprets results	Aflibercept must be held at least four weeks before elective surgery; oncology directs timing
Heart assessment	Review cardiac history; baseline ECG if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
international normalised ratio (blood-clotting test) (INR) only if taking warfarin	Tell the team about warfarin; arrange INR monitoring	INR as clinically indicated, rather than a universal weekly schedule	Report bleeding, bruising or significant changes in intake	The prescribing team adjusts warfarin if needed	Continue the individually agreed anticoagulation follow-up

Check	Before starting	During treatment	Between visits	Review / change	After treatment
Response / scans / tumour markers	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00238 v6 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Aflibercept + FOLFIRI: treatment and checks

Metastatic colorectal cancer

Day 1: aflibercept, then irinotecan, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. Repeat every 14 days while beneficial and tolerable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver NCCP 00238	Required baseline blood count, kidney and liver tests	Required before each cycle; results reviewed before treatment	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
DPD / fluoropyrimidine safety NCCP 00238	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
Blood pressure / urine / healing NCCP 00238	Required baseline BP and urine protein	Urine dipstick each cycle; blood pressure monitored during treatment	Report severe headache, bleeding, swelling, reduced urine or poor healing	A raised urine protein/creatinine ratio may require a 24-hour collection; team interprets results	Aflibercept must be held at least four weeks before elective surgery; oncology directs timing
Heart assessment NCCP 00238	Review cardiac history; baseline ECG if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
INR only if taking warfarin NCCP 00238	Tell the team about warfarin; arrange INR monitoring	INR as clinically indicated, rather than a universal weekly schedule	Report bleeding, bruising or significant changes in intake	The prescribing team adjusts warfarin if needed	Continue the individually agreed anticoagulation follow-up
Response / scans / tumour markers NCCP 00238	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

What each check is for

1 14-day treatment cycle

Day ward → home pump → review

Aflibercept with FOLFIRI is used for metastatic colorectal cancer that is resistant to, or has progressed after, an oxaliplatin-containing treatment. The aim is to control the cancer and symptoms; treatment continues while its benefit outweighs its side effects.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

Sources: NCCP 00238

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.33